ONE Clinical Sensory Integration Protocol

POSSIBLE F-TAG/QMS

Resident Rights

- Self-Determination

Quality of Life

- ADL Maintenance
- Activities meet Interest/needs

Behavioral Health

- Sufficient/Competent Staff-Behavioral Health Needs
- Treatment/Svc for Mental/Psychosocial
- No Pattern of Behavioral Difficulties Unless Unavoidable
- Treatment/Service for Dementia

Pharmacy

- Drug Regimen is Free from Unnecessary Drugs
- Free from Unnecessary Psychotropic Meds/PRN Use

THERAPIES

- PT
- OT
- SLP

PURPOSE OF THERAPY

- Increase the patient's awareness of their body in space and improve their ability to navigate their environment safely and efficiently.
- Improve the patient's ability to attend to and process sensory information, such as touch, sound, and visual cues.
- Specific individualized sensory stimulation used throughout the day to accentuate remaining abilities and increase both comfort and engagement
- Manage sensory modulation problems (e.g., touch, movement, visual or auditory sensitivity) and related emotions and behaviors, such as anxiety or self-injury, to help change sensory processing patterns, minimize crisis escalation, or promote calming for overall health and wellness
- Enhance the patient's ability to organize and coordinate their movements in response to sensory input

GOAL EXAMPLES

OT:

- Pt will demonstrate an improvement in their ability to attend to and process sensory information by completing at least 80% of sensory-rich activities or ADLs provided by the therapist.
- Pt will demonstrate an improvement in their ability to navigate their environment safely and efficiently by completing at least 80% of the balance and coordination exercises provided by the therapist.
- Pt will safely and effectively attend to self-feeding activities for 10 minutes following sensory activity.
- Pt will require less than 25% redirection back to task to continue sustained attention tasks when allowed 10 minutes of sensory activity.
- Pt will identify 2 sensory strategies during ADL care to promote engagement and comfort without episodes of behavioral outburst.

TOOLS & ASSESSMENTS

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- ·Use of standardized tests, such as the Sensory Profile, to assess the patient's sensory processing skills
- Adult sensory Motor checklist
- Sensory Diet Exploration check list activity
- Assessment of comfort vs discomfort during ADLs including meal time, bathing, dressing, ROM, dressing and grooming to determine best areas to focus on increasing comfort
- Dementia Behavior Disturbance Scale: utilized to measure dementia behaviors including repetitive questions, losing or hiding things, lack of interest in activities, nocturnal wakefulness, unwarranted accusations, excessive daytime sleeping, and other behaviors often associated with sundowning.
- Cognitive Assessment Tool Guide and associated Allen Cognitive Levels to stage cognition and develop care considerations by level.
- Use of functional assessments, such as the Occupational Performance History and Routine Task Inventory, to assess the patient's ability to engage in daily living activities.
- Use of observational assessments, such as the Sensory Processing Measure, to assess the patient's ability to attend to and process sensory information.

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POSSIBLE F-TAG/QMS	THERAPIES	PURPOSE OF THERAPY	GOAL EXAMPLES	TOOLS & ASSESSMENTS
Quality Measures • Antipsychotic Medications/Behavior Symptoms Affecting others: medications are used most often to decrease behavioral symptoms; can lead to: increased cost for medications, can lead to increased confusion, can lead to need for increased nursing supervision (QM category: Received Antipsychotics Medication) • Antianxiety/Hypnotic/Depression Symptoms: medications are used most often to decrease depressive/anxiety symptoms; can lead to: increased cost for medications, can lead to increased confusion, can lead to need for increased nursing supervision due to drowsiness/decreased participation in therapy, can lead to increased social isolation (QM category: Received Antipsychotics Medication) • Falls with Major Injury		• Increase the patient's ability to use their senses to engage in daily living activities, such as dressing, grooming, and eating. • Decrease areas of discomfort or agitation during daily routines to accentuate remaining abilities and promote successful participation in productive activities. • Decrease symptoms of sensory processing disorders to allow for safe mobility within SNF/ALF/ILF to decrease risk of touching, bumping in others, and pacing.	**ST:** • Pt will respond to simple communication and stimulation via head turns, visual tracking, and vocalizations with 60% acc. given tactile/verbal cues to improve socialization and reduce confusion during ADL completion. • Pt will appropriately respond to redirections/adaptation techniques with 75% acc given mod cues in order to engage pt in increasingly meaningful interactions with family/staff and in order to promote optimal performance with ADLs and activities of choice. • Pt will respond to verbal/visual/tactile stimulation in 5/10 presentations given max multi-modality cues in order to improve meaningful interactions with others and decrease risk of social isolation. • Pt will attend to simple multisensory tasks for ~3 minutes in 7/10 attempts with max multi-modality cues to improve ability to participate in meaningful interactions and/or tasks. **PT:** • Patient will respond to various forms of sensory stimulation included but not limited to (tactile stimuli, responds/localizes to sounds, tracks movement) in 3/5 opportunities to increase alertness for functional task performance.	• Use of accommodations and adaptations like wearing noise cancelling headphones to improve attention, use of loofah sponge during bathing for hypersensitivity, etc. • Use of environmental modifications toward lighting, sounds such as white noise machines, or other equipment that can increase or decrease sensory stimulation a space provides. • Use of remedial interventions such as activities that encourage engagement of tactile, proprioceptive, and movement opportunities and promote motor planning, such as cooking or meal prepping activities. • Use of sensory diet programs that are individualized such as rocking chair, quiet space, aromatherapy, weighted blanket, stress balls, fidget items, etc. to help manage sensory modulation problems or reduce symptoms of anxiety or agitation to encourage participation in needed or desired activities. • Visual stimulation can include calming images, auditory stimulation can include relaxation/environmental music, olfactory stimulation can include aromatherapy, essential oils, and familiar scents, tactile stimulation can include texture variations, busy boards and functional fine motor activities, and proprioceptive stimulation can include items such as rocking chairs to improve communication, meaningful interactions, and reminiscing.

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			Patient will develop a systematic scanning technique to localize and identify stationary objects at tabletop with L hand 50% of the time to increase in hand manipulation and increase participation in ADLs. Patient will be able to scan in horizontal plane, requiring min verbal for convergence of eye movements, and fast approaching objects. Patient will identify and implement stress management skills (breathing, music, stretching, visual imagery, relaxation techniques) to promote sleep and decreased anxiety with min A to recall and initiate ability to sleep 90-100% of the time. Using a sensory diet and environmental modifications, patient will participate in therapeutic activities with Min signs and symptoms of anxiety (including yelling/calling out, hitting, O2 drops, etc.) while increasing attention to task × 15 mins with Mod cues for attention/redirection.	